

Workout Injury Risk Report

2026-09-29 10:41

Health Summary

BMI 19.8 (Normal) - Goal: Bulk

AI health summary unavailable for this report.

Workout Summary

Duration: 01:55

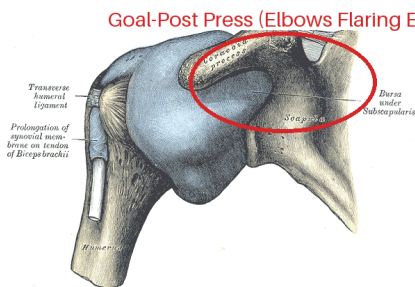
Exercises Completed

- Shoulder Press: 8 reps
- Bicep Curls: 9 reps
- Pulldown: 15 reps

Mishaps & Injury Risk

AI summary unavailable - showing the standard explanations. The advice and sources below are unchanged.

Goal-Post Press (Elbows Flaring Early)



Evidence: Based on an official training guideline (US Army FM 7-22)

Your elbows swung out to the sides before your arms had straightened, so for much of the rep your shoulders held the weight out wide instead of pushing it straight up. The press partly turns into a lateral raise under a heavy load. FM 7-22's overhead press starts with the shoulders directly below the hands and pushes straight up.

How to prevent it

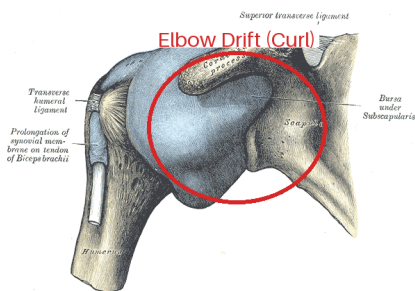
Fix it now: Keep your forearms vertical, with your wrists stacked over your elbows, and push the weight in a straight line up over your shoulders.

Corrective drill: Shoulder Stability Drill (FM 7-22, p. 6-25): I, T, Y, L and W raises lying face down, 5 slow reps each, bodyweight only. FM 7-22 uses it to build shoulder strength and stability, and in rehab after shoulder injuries.

Progress safely: Lighten the load until the bar or dumbbells travel straight up on every rep. Add weight in small steps and only after a full session of clean reps. Injury risk rises with the size of the jump from what your body is used to (HHS, p. 90).

Sources: US Army FM 7-22 (2012), p. 6-47, overhead press starting position; Kolber et al., J Strength Cond Res (2010), technique as a shoulder-injury risk factor

Elbow Drift (Curl)



Evidence: Technique standard - no specific injury study; general shoulder-injury evidence applies

Your elbow drifted forward and away from your side during the curl, so the front of your shoulder started helping to lift the weight. This is mainly a technique fault: the biceps does less of the work, and the shoulder, the most commonly injured area in weight training, picks up load it doesn't need to carry. No study has linked this specific fault to injury, so treat it as technique coaching.

How to prevent it

Fix it now: Pin your elbows to your sides and move only your forearms. Use a weight you can lift without swinging.

Corrective drill: Curl at a slow, controlled pace, as in FM 7-22's machine biceps curl (p. 6-62), with no arching back to move the weight.

Progress safely: If you have to swing your body or let your elbows drift to finish a rep, the weight is too heavy. Drop it until every rep is clean.

Sources: US Army FM 7-22 (2012), p. 6-62, biceps curl; Kolber et al., J Strength Cond Res (2010), general shoulder-injury evidence

General Injury Prevention

- Increase load gradually. Injury risk to bones, muscles and joints is tied to the gap between what you usually do and what you do next. Make small increases and give your body time to adapt. (HHS Physical Activity Guidelines (2018), p. 90)
- Warm up before strength work by doing your exercises with lighter weight first. (HHS Physical Activity Guidelines (2018), p. 91)
- More is not better. Soreness and fatigue that never fully clear, together with falling performance, are early signs of overtraining. Rest, recovery and good nutrition bring more improvement than training harder. (US Army FM 7-22 (2012), p. 5-3)
- If you have been injured before, that body part is at higher risk of injury again, so progress it more slowly than the rest. (HHS Physical Activity Guidelines (2018), p. 89 and 91)

References

- [1] Headquarters, Department of the Army. FM 7-22: Army Physical Readiness Training. Washington, DC; October 2012 (incl. Change 1, May 2013). (Government guideline)
- [2] Kolber MJ, Beekhuizen KS, Cheng MS, Hellman MA. Shoulder injuries attributed to resistance training: a brief review. J Strength Cond Res. 2010;24(6):1696-1704. doi:10.1519/JSC.0b013e3181dc4330 (Peer-reviewed review)
- [3] U.S. Department of Health and Human Services. Physical Activity Guidelines for Americans, 2nd edition. Washington, DC: HHS; 2018. Chapter 7, Active and Safe. (Government guideline)

This report gives general exercise-technique guidance based on the sources above. It is not a medical diagnosis. If you have pain, especially sharp or lasting pain, stop the exercise and see a doctor or physiotherapist.